

The Side Effect Survival Sheet

What's normal, what helps, and what actually needs a phone call.

FROM A PHYSICIAN ASSISTANT WHO'S BEEN ON ONE HERSELF

Most of what people panic about is expected and manageable. A few things are not, and those are the ones nobody tells you to watch for. So the red flags come first here, before any of the comfort tips — that order is deliberate.

CALL SOMEONE ABOUT THESE. DON'T WAIT IT OUT.

Severe stomach pain that bores through to your back, especially with vomiting that will not stop. This is the one people push through and shouldn't.

Vomiting or diarrhoea you can't keep ahead of — not urinating, dizzy on standing, heart racing. Dehydration is the most common reason people end up in an ER.

Pain in your upper right side, particularly after fatty food, sometimes with your skin or eyes looking yellow.

A new lump in your neck, trouble swallowing, or a hoarse voice that sticks around.

Vision changes. Blurring or a sudden shift in how well you see.

Anything that frightens you. You are allowed to call and be told it's fine.

None of that is a reason to be afraid of the medication. It's a reason to know which door to knock on, and when.

The common ones, and what actually helps

NAUSEA

The mechanism explains the fix. These medications slow how fast your stomach empties, so a large fast meal lands in a stomach that's already behind. Most nausea is volume and speed, not the food itself.

- **Smaller plates, more often.** The single change that helps most people most.
- **Slow down.** Fullness arrives late now. Eat at your old speed and you'll sail past it.
- **Protein first, then vegetables, then the rest.** Same meal, different order.
- **Stop at satisfied, not full.** Full is already too far on these.
- **Go easy on very fatty or fried meals** — they sit longest.
- **Cold food often goes down easier than hot** when nothing sounds good.

CONSTIPATION

Extremely common and mostly a side effect of eating far less than you used to, rather than of the medication itself. Less food in means less moving through.

- **Water first, before fibre.** Fibre without fluid makes it worse, not better.
- **Move a little, daily.** A walk counts. It genuinely helps.
- **Fibre from food where you can manage it** — but see the water line above.
- **If it's been days, ask your provider what they want you to use.** Don't improvise from a forum.

FATIGUE

Usually the boring explanation: not enough food, not enough fluid, or not enough protein. Worth ruling those out before assuming it's the medication.

- **Are you actually eating?** Appetite suppression makes skipping meals effortless.
- **Are you drinking?** Thirst cues drop along with hunger cues.
- **Is protein anywhere near where it should be?**
- **Ask about your labs.** Iron, B12 and thyroid are worth a conversation if it persists.

The things nobody warns you about

Food stops sounding good — including food you love

This surprises people more than the nausea does. It's normal, and it's also the reason under-eating sneaks up. Eat on a schedule rather than on appetite, at least at first.

Alcohol can hit differently

Less food, a slower stomach, and often less interest in drinking at all. If you do drink, treat your old tolerance as out of date.

Your hair may shed a few months in

Usually tied to rapid change and to intake dropping, not to the medication attacking your hair. It is worth checking rather than guessing — I have a separate free guide on what to ask about before you buy anything.

Muscle goes along with fat unless you defend it

Protein and some resistance work, a couple of times a week. Carrying groceries counts more than nothing.

The pattern worth learning: most side effects answer to **smaller, slower, sooner** — smaller portions, slower eating, and asking sooner rather than waiting to see. **The red-flag list on page one is the exception**, and it does not wait.

I'm a physician assistant, I've been on a GLP-1 myself, and the questions I get most are about side effects — not results.

Come find me: Instagram @diary_of_the_most_okayest_mom — I answer questions there, and the side-effect stuff nobody warns you about is most of what I post.

Educational only. This is not medical advice and it isn't a substitute for your own provider's judgment. It does not cover every possible side effect, and it is not a reason to start, stop, or change any medication. Individual experiences vary; only a licensed provider who knows your history can advise you.

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